Whitaker Physical Therapy

0607 Michelle Ports
Conroe, TX
Phone 346-555-6416

Itemized Medical Bill

Patient:	Edward Rogers
Date of birth:	12 Mar 1951
Insurance claim:	C-1495
Date of injury:	9 Jan 2024
First visit:	12 Jan 2024
Last visit:	10 May 2024
Number of visits:	21

Services

Service	Units	Charge
99203 New patient evaluation	1	$286.36
98941 Spinal manipulation	5	$560.80
97110 Therapeutic exercise	6	$1,083.94
97124 Massage therapy	2	$173.71
72040 X-ray, cervical spine	2	$1,052.67
20552 Trigger point injection	5	$2,855.38
TOTAL		$6,012.86

Recent visits

Date	Service	Charge
9 Apr 2024	97110 Therapeutic exercise	$184.98
16 Apr 2024	98941 Spinal manipulation	$108.82
20 Apr 2024	98941 Spinal manipulation	$131.73
24 Apr 2024	98941 Spinal manipulation	$95.95
1 May 2024	20552 Trigger point injection	$579.60
10 May 2024	72040 X-ray, cervical spine	$503.52

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Treatment History and Summary

Patient:	Edward Rogers
Treating provider:	Barbara Harrell, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
January 2024: evaluation, pain management	3	$1,516.09
February 2024: massage therapy, physical therapy	4	$604.02
March 2024: chiropractic adjustment, imaging, massage therapy, pain management, physical therapy	6	$1,479.64
April 2024: chiropractic adjustment, pain management, physical therapy	1	$1,329.99
May 2024: imaging, pain management	2	$1,083.12

Treatment summary

Patient presented following a sideswipe collision that occurred during a lane change, reporting right knee pain and swelling. The mechanism of injury is consistent with the reported force of the impact, and the pattern of symptoms matches what would be expected from this type of event. Over the course of 21 visits, care followed a conventional progression, beginning with rest, ice, and anti-inflammatory measures, then moving to supervised physical therapy focused on range of motion and strengthening. Visit frequency decreased steadily as symptoms improved and function returned. Physical examination revealed localized tenderness and a limited range of motion, which support the reported injury, while several ligament stability tests and neurovascular checks returned normal results. The patient reports no prior injury or treatment involving this knee. Billing records reflect single, appropriate charges for each service rendered, with no duplicated entries. At discharge, the patient was ambulating without assistance and was released to full activity with a home exercise program.